M5 Atropine

Scope of Practice	EMT-A
Classification	• Anticholinergic
Usual Presentation	• 0.5mg/5ml (prefilled syringe)

Mechanism of Action	• Atropine blocks muscarinic acetylcholine receptors reducing vagal tone.
	• This results in:
	  • Increased heartrate
	  • Drying of salivary and bronchial secretions
	  • Bronchodilation

Indications	• Compromised Bradycardia. • Organophosphate poisoning.
Contraindications	• Known allergy
Precautions	• Myocardial Ischemia – Atropine will increase myocardial oxygen demand. • Glaucoma – Atropine will increase intraocular pressure.
Side Effects	• Tachycardia and Palpitations • Confusion • Dry mouth • Blurred vision
Adult Dose & Administration	**Bradycardia** • 1mg IV every 3-5 minutes. **Organophosphate Poisoning** • 2mg IV Initial Dose • Double each subsequent dose (e.g. 4mg/6mg/8mg) until resolution of symptoms.
Pediatric Dose & Administration	**Bradycardia** • 0.02mg/kg IV/IO every 3-5 minutes. **Organophosphate Poisoning** • 0.02-0.05mg/kg IV/IO every 3-5 minutes.
Use in Pregnancy and Lactation	• Pregnancy: Fetal harm has been demonstrated. However, atropine should not be withheld if the patient is significantly compromised. Transcutaneous pacing is preferred if available. • Lactation: Infant harm cannot be ruled out. Atropine may be administered, the mother should be advised to stop breast feeding until advised by their physician.
Additional Information	• Administer as a fast IV push. • Slow administration may result in further (transient) bradycardia.